10.2 Sickle Cell Crisis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain in my bones is unbearable." • Rates pain 10/10 **Objective** • Known sickle cell disease • Guards his limbs • HR 120/min	**ACTUAL** Acute pain related to vaso-occlusion and tissue ischemia as evidenced by severe bone pain.	• Pain bearable within an hour of analgesia.	• Give strong analgesia without delay, as prescribed (often opioids per protocol). • Reassess pain every 30–60 minutes. • Apply warmth to painful areas; avoid cold. • Accept his own report of pain.	• Considered met when pain is tolerable.
Subjective • "I am thirsty." **Objective** • Dry mouth • Concentrated urine	**ACTUAL** Inadequate fluid volume related to inadequate intake and increased losses as evidenced by dry mucosa.	• Well hydrated within 24 hours.	• Give IV and oral fluids as prescribed. • Keep an intake and output chart. • Give oxygen if SpO_2 is below target.	• Goal reached once he is hydrated.
Subjective • "I have chest pain and cough." **Objective** • Sickle crisis • Shallow breathing limited by pain	**RISK** Risk for infection as evidenced by risk factors: functional asplenia and pulmonary involvement (acute chest syndrome).	• Acute chest syndrome and infection picked up early.	• Follow SpO_2, RR and temperature. • Encourage incentive spirometry. • Report chest pain, fever or a fall in SpO_2 at once.	• Met if no complications develop.

*References for this care plan: [9], [32], [35], [38]. Full entries are listed in the References section of the CPMN book.